Depressive and Bipolar Disorders ✕

Today's goal • 3/3 items

Module 1
Learn With PsycLearn Essentials

Start Your Course Here

✓ A Welcome Video From Your Instructor
Video • 2 min

✓ Welcome to PsycLearn Essentials
Reading • 3 min

○ Survey: What Would You Like to See Next?
Reading • 2 min

✓ How to Earn Certificates
Reading • 2 min

Course Overview

✓ What's in Your Course
Reading • 5 min

Study Success in PsycLearn

✓ Study Tips for Success in PsycLearn Essentials
Reading • 5 min

Additional Course Information

✓ Additional Information
Reading • 10 min

✓ Representation in Research
Reading • 1 min

Module 2
Opening to Depressive and Bipolar Disorders

Lesson 1: Culture and Mood Disorders

✓ Focus of the Course
Reading • 2 min

✓ Classifying Psychopathology
Reading • 4 min

○ **Culture and Psychopathology**
Reading • 4 min

○ Culture and Intersectionality
Reading • 4 min

○ Culture and Stigma
Reading • 4 min

○ Check Your Understanding: Culture and Mood Disorders
Practice Assignment • 5 min

Lesson 2: Introduction to Depressive and Bipolar Disorders

○ Depressive and Bipolar Disorders
Video • 6 min

○ Living with a Mood Disorder: A Case Study
Reading • 10 min

○ Overview of Depressive and Bipolar Disorders
Reading • 10 min

○ Check Your Understanding: Introduction to Depressive and Bipolar Disorders
Practice Assignment • 5 min

Key Takeaways

○ Key Takeaways: Opening to Depressive and Bipolar Disorders
Reading • 10 min

Module 2 Quiz

○ Module Quiz: Opening to Depressive and Bipolar Disorders
Graded Assignment • 10 min

Module 3
Depressive and Bipolar Disorders

Module 4
Biological Contributing Factors

Module 5
Psychological Contributing Factors

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Treatments for Depressive and Bipolar Disorders

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Conclusion

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Culture and Psychopathology

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Culture is the distinctive customs, values, beliefs, knowledge, art, and language of a society or a community that are the basis for everyday behaviors and practices, passed on from generation to generation, sometimes overtly but often implicitly. In short, culture is the societal norms and expectations that guide individual behavior. In addition, people both affect and are affected by their culture. Overall, culture emerges from daily life and interactions between people (López & Guarnaccia, 2016).

Researchers have found differences in psychopathology across the world. Recognizing the changing face of the culture of mental health, *ICD-11* includes "culture-related features" as part of its diagnostic descriptions for mood disorders, which includes depressive disorders, dysthymic disorder, bipolar disorders, and cyclothymic disorder. These features focus on the idea that different cultures may show the same distress (e.g., depression) in different ways. For example, depressive disorders in Asian cultures (e.g., China, South Korea) may appear as physical symptoms, including headache, insomnia, and dizziness, whereas psychological symptoms, predominantly depressed mood, are more common in European-American cultures (Ryder et al., 2008). In addition to these differences, some cultures may describe depressive symptoms in moral (e.g., guilt or worth), social (e.g., relationship conflicts), hedonistic (e.g., reduced pleasure seeking), or even spiritual (e.g., messages in dreams) terms.

Researchers Laurence Kirmayer and Andrew Ryder (2016), among others, have suggested that cross-cultural research that compares the population of one country or region to another is becoming less valuable. Rather, they recommend focusing on subcultures, where the influences of culture can be more accurately assessed and compared.

One example is a study that examined the prevalence of psychotic experiences in groups of ethnic minority youth in the Netherlands (Adriaanse et al., 2015). Rather than simply lumping all ethnic minority members together and comparing them to the majority, the researchers split the ethnic minority youth into four groups, based on the country of origin of their parents or grandparents. Their minority participants consist of children who were born in the Netherlands and are of Moroccan descent, of Turkish descent, from a different country with either a low or middle average income, or from a different country with a high average income. These four groups were compared to children whose parents and grandparents were born in the Netherlands.

For members of each group, the researchers measured the prevalence of psychotic experiences, like delusions and hallucinations, and the degree to which these experiences significantly impacted their lives. As they expected, they found that minority groups overall had more psychotic experiences than the Dutch-majority children. However, because they broke down minority groups by country of origin, their conclusions went further. For example, they showed that the Turkish-Dutch children had a higher proportion of auditory hallucinations than did the other children, whereas the children whose parents or grandparents immigrated from a high-income country other than Morocco or Turkey had a higher incidence of believing that others could read their mind.

The finding that different ethnic subgroups present different profiles of psychotic symptoms is an important step toward developing a more nuanced understanding of the way culture can affect psychological health.

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